

How to Open an In-Home Care Agency in Oregon

A step-by-step guide to getting licensed by the Oregon Health Authority — choosing your class, screening and training caregivers, and getting paid

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The Oregon difference — read this first

Oregon licenses in-home care, and it does so in four classes. Unlike states that leave non-medical home care unlicensed, Oregon requires an In-Home Care Agency (IHCA) license from the Oregon Health Authority before you serve a single client — and the license comes in four classes (Limited, Basic, Intermediate, and Comprehensive) that set exactly what services you may provide, from personal care up through medication and nursing services. Choosing the right class is the first big decision you make. This guide walks you through the license and class, the caregiver screening, registry, and training Oregon requires, and how you get paid — privately and through the Oregon Health Plan. Oregon also has no statewide sales tax, a small but real edge in a premium care market.

How to Use This Guide

This guide takes you from idea to a licensed, operating Oregon in-home care agency. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Oregon agency owners — not legal, tax, or licensing advice. Requirements, fees, and phone numbers change, so verify each step with the agency named before you rely on it. Where this guide gives a fee or a figure, treat it as a starting point to confirm, not a final answer.

The one-paragraph version

Form an Oregon business and get an EIN. Decide which of the four license classes matches the services you will provide — Limited, Basic, Intermediate, or Comprehensive — because the class sets your scope and your fee. Put your insurance and a qualified administrator in place, adopt your policies, and apply to the Oregon Health Authority through its licensing portal at least 60 days before you intend to open. Build your caregiver pipeline: every caregiver clears an Oregon background check, is on the caregiver registry, completes at least four hours of orientation and initial training matched to your services, and has their competency evaluated; caregivers who provide medication services complete medication training too. Pass the OHA survey, get your license, and start serving clients — privately at first, then through the Oregon Health Plan once you enroll as a Medicaid provider, which is a separate step from your license.

The Path at a Glance

Ten steps take you from idea to an operating Oregon in-home care agency:

- 1. Choose your class.** Limited, Basic, Intermediate, or Comprehensive — the class sets which services you may provide and your application fee.
- 2. Form the business.** Register an LLC or corporation with the Oregon Secretary of State; get an EIN; open a business bank account.

3. **Name a qualified administrator.** A person who meets the Division 536 administrator qualifications to run the agency.
4. **Put protection in place.** Professional and general liability insurance and workers' compensation coverage.
5. **Write your policies.** Adopt the policy and procedure manual, client forms, and caregiver handbook the license requires.
6. **Apply to OHA.** Submit your application and class fee through the licensing portal at least 60 days before opening.
7. **Build caregiver screening & training.** Background checks, the caregiver registry, orientation, initial training, and competency evaluation.
8. **Pass the survey.** OHA reviews your application and surveys the agency before issuing the license.
9. **Start serving clients.** Begin with private pay; deliver, document, and supervise every visit per the service plan.
10. **Add Medicaid and grow.** Enroll as an Oregon Health Plan / APD provider, and build referral relationships across your region.

Part 1 — What This Business Is

An in-home care agency sends caregivers into clients' homes to help with everyday life: bathing, dressing, grooming, toileting, transferring, and eating (personal care); cleaning, laundry, meals, and errands (household and supportive services); and, depending on your license class, help with medications and even nursing services. In Oregon this is a licensed activity — you are not just starting a business, you are opening a state-licensed care agency, and that shapes everything from how you hire to how you get paid.

Oregon is a strong and growing market. The state has a large and aging population, a strong preference for keeping older adults at home, and both a premium private-pay market and a Medicaid program (the Oregon Health Plan) that funds in-home care. Oregon also has no statewide sales tax, which simplifies pricing. The trade-off for entering a licensed market is real: you must clear a genuine licensing bar before you open, and you operate under ongoing survey — but that same bar keeps out unserious competitors and gives licensed agencies credibility with the hospitals, case managers, and families who choose them.

Practically, being licensed changes how you run day to day. You cannot quietly start with one client and figure out compliance later — the license comes first, and everything that supports it (screened and trained caregivers, written service plans, supervision, and records) has to exist before you open. That feels like more work up front, and it is, but it also means the systems that protect your clients and your business are built in from day one rather than bolted on after a problem. Think of the license as the operating discipline that lets you scale: an agency that is genuinely survey-ready can add caregivers and clients without the wheels coming off.

Is this business right for you?

Opening an in-home care agency in Oregon rewards people who are organized, patient with paperwork, and genuinely good with people — because the job is equal parts care and compliance. You do not need a clinical background to own a Limited, Basic, or Intermediate agency, though you do need a qualified administrator and, for Comprehensive, a registered nurse. You do need enough capital to cover several months of licensing and startup before revenue arrives, the temperament to recruit and retain caregivers in a competitive labor market, and the discipline to keep records a surveyor could open at any time. If that sounds like you, Oregon's licensed model is an advantage, not an obstacle: it keeps casual competitors out and rewards operators who take the work seriously.

The service lines, in plain terms

- **Personal care:** hands-on help with the activities of daily living — available at every license class.
- **Household and supportive services:** housekeeping, laundry, meals, shopping, and errands that keep a home safe and livable.
- **Medication services:** medication reminding, assistance with self-administration, medication set-up, and (at the top class) administration — available by class.
- **Nursing services:** nursing assessment, monitoring, and delegation by an Oregon-licensed registered nurse — available only at the Comprehensive class.

Part 2 — The License and Choosing Your Class

Here is the single most important thing to understand about opening in Oregon: in-home care is licensed by the Oregon Health Authority (OHA) under Oregon Administrative Rules Chapter 333, Division 536, and the license comes in four classes. The class you choose defines the services you may legally provide and sets your application fee, so choose it deliberately.

Class	Application fee	What it lets you provide
Limited	\$2,000	Personal care and household and supportive services
Basic	\$2,250	The above plus medication reminding and medication assistance
Intermediate	\$2,500	The above plus medication set-up (into secondary containers)

Class	Application fee	What it lets you provide
Comprehensive	\$3,000	The above plus medication administration and nursing services

Fees are the initial application fees by class; verify current amounts with OHA. The class also drives your training and staffing obligations.

A Limited class is the fastest and cheapest to enter and the simplest to run, but it cannot touch medications — which many clients need. A Comprehensive class can serve the widest range of clients, including those needing nursing oversight, but it must employ or contract a registered nurse and carries the heaviest training and documentation load. Most new agencies choose the class that matches the clients they intend to serve and the team they can actually resource, and apply to change class later as they grow. Delivering a service above your class is operating outside your license — so if you plan to help with medication, license at Basic or above from the start.

A useful way to choose your class is to picture the client you most want to serve. If your niche is companionship, personal care, and household help for older adults who manage their own medications, Limited may be enough. If your clients need reminders and hands-on help taking medication they self-direct, you need Basic. If you want to organize medications into daily or weekly containers, you need Intermediate. And if you intend to serve clients who need medication actually administered or nursing oversight — a growing, higher-value part of the market — you need Comprehensive and a registered nurse. It is easier to license at the class you will grow into within a year than to reapply repeatedly, but do not license Comprehensive “just in case” if you are not ready to staff the nursing and training it requires.

The mindset that wins in a licensed state

Operate to the rule, and let the license work for you. In a licensed market, your license and a clean survey record are marketing assets — hospital discharge planners, case managers, and families check them before they refer or choose. Build your screening, training, supervision, and documentation to the Division 536 standard from day one, and the license becomes the thing that sets you apart from unlicensed help.

Part 3 — Getting Licensed by OHA

You apply for your In-Home Care Agency license through the Oregon Health Authority's licensing portal. Plan for the process to take time — a realistic estimate is several months from a complete application to an issued license, once you account for review, background checks, and the survey.

What the application involves

- **Apply at least 60 days ahead.** Submit your application and the class-based fee through the OHA licensing portal at least 60 days before you intend to open.
- **Include your policies and administrator.** Your application includes your policies and procedures, evidence of a qualified administrator, background-check authorizations, and proof of insurance.
- **Pass the survey.** OHA reviews your application, processes background checks, and conducts a survey. At survey, you show your policies, at least one complete caregiver record proving screening, training, and competency, and mock client files as OHA directs.
- **Get your license.** OHA issues the license after a passing survey — often within about ten business days of passing. You do not accept clients or provide services before the license is issued.
- **Respond to deficiencies.** If the survey finds gaps, respond promptly with corrections; a complete, accurate application from the start is the best way to avoid delays.

Be realistic about the timeline. Between assembling a complete application, the 60-day lead time, background-check processing, scheduling and passing the survey, and clearing any deficiencies, several months is normal — some agencies take the better part of a year from first paperwork to first client. Use that time productively: it is exactly when you build your caregiver pipeline, write and refine your policies, and line up your first referral relationships, so that the day your license issues you are ready to take clients rather than starting from zero. The agencies that open fastest are the ones whose application, policies, and caregiver files all tell the same consistent, complete story.

Use OHA's own checklist

OHA provides application and survey-preparation resources, including a matrix or checklist that maps your policies to the current rules. Building your policy manual and your caregiver and client files against that checklist before you submit is the single best way to avoid deficiency notices and shave time off your licensure. Renew your license on time — the renewal application is due before expiration — and keep the agency survey-ready at all times, because OHA can survey you or investigate a complaint after licensure, not just at the start.

Part 4 — Caregiver Screening & the Registry

Your caregivers are your business, and Oregon sets specific requirements for who may provide care. Get this right and everything else follows; get it wrong and it is both a survey finding and a safety risk.

- **Criminal background checks.** Every owner, administrator, and caregiver completes an Oregon criminal records check through the Background Check Unit and receives an approved fitness determination before providing services or having client contact. No approval, no client contact — no exceptions.
- **The caregiver registry.** Caregivers are enrolled in the applicable Oregon caregiver registry, and you confirm status before assigning care and monitor it over time.
- **Age and fitness.** Caregivers are at least 18, and at least 21 to work alone with a client for extended periods, and are able to perform their assigned tasks safely.
- **Credential verification.** You verify any Oregon health-care license or certificate a caregiver holds — which also exempts them from in-home caregiver training, though you still evaluate their competency.

Because background checks take time, start them the moment you make a conditional offer, and keep a simple tracker for every caregiver showing background, registry, training, and competency status — so you can prove, at a glance, that no one served a client before they were ready.

Recruiting is the constant work of this business, so treat your caregiver pipeline as a system, not a scramble. Advertise steadily, respond to applicants fast, and move good candidates through offer, background check, registry, orientation, training, and competency without letting them cool off — the best caregivers have options. Pay competitively for your region, treat people well, and build a culture caregivers want to stay in, because every caregiver you keep is one you do not have to recruit, screen, and train again. A reliable bench of screened, trained, competency-evaluated caregivers is what lets you say yes to a new client or referral on short notice, which is exactly what referral sources remember.

Part 5 — Orientation, Training & Competency

Oregon requires you to prepare and evaluate every caregiver before they work independently, and the requirements scale with the services you provide.

- **Orientation — at least four hours.** Before independently serving clients, each caregiver completes an agency-specific orientation of at least four hours, covering your policies, client care practices, clients' rights, abuse reporting, and the rules for traveling with a client.
- **Initial caregiver training.** Training matched to the services you provide — safe personal-care techniques, preventing skin breakdown, contractures, and falls, assisting with self-directed medication, cultural competence, and population-specific topics such as dementia care.
- **Medication-services training — at least four hours.** Caregivers who provide medication services at a Basic, Intermediate, or Comprehensive agency complete at least four hours of basic non-injectable medication training before providing them; Intermediate and Comprehensive add medication set-up.
- **Competency evaluation.** Every caregiver's competency is evaluated by a combination of direct observation and written or oral testing before they serve independently, and re-evaluated for new tasks or populations.

Document everything, in every file

Oregon's training and competency requirements are only as good as your records. Document each caregiver's orientation, initial and medication training (content, date, length, and instructor), and competency evaluation in their personnel file, and keep proof that any trainer was qualified. At survey, OHA will pull a caregiver file and expect to see that the person was screened, trained, and found competent before they ever served a client.

Training is also where you win or lose in Oregon's tight caregiver labor market. The agencies that keep good caregivers are the ones that make orientation and training feel like real preparation and respect rather than a box to check — a caregiver who starts confident and supported is safer, stays longer, and costs you less in turnover. Build your training so it genuinely prepares people for the homes and clients they will serve, evaluate competency honestly, and keep investing in your caregivers after hire; it is both a compliance requirement and your best retention strategy.

Part 6 — Setting Up to Operate

Form the business and name an administrator

Register an LLC or corporation with the Oregon Secretary of State, get a federal EIN, and open a business bank account. Name a qualified administrator who meets the Division 536 qualifications and has the authority and time to actually run the agency — OHA holds the administrator accountable, so this is not a name on paper. Oregon has no statewide sales tax, but you still meet all Oregon and federal tax and employment obligations.

Insurance, policies, and supervision

Carry professional and general liability insurance and workers' compensation coverage before any caregiver starts. Adopt a policy and procedure manual, client forms, and a caregiver handbook built to Division 536. Assess each client, write a service plan, review it with each assigned caregiver before care begins, and supervise service delivery — the service plan and supervision are your quality-control system and a survey checkpoint.

Your administrator is the linchpin. OHA holds this person accountable for the agency, so give the role real authority and time — an administrator who is stretched too thin or exists only on paper is both a compliance risk and the reason quality slips. The administrator owns your compliance calendar (license renewal, background-check re-checks, training currency, insurance, and business filings), your service-planning and supervision systems, and your quality program that tracks complaints, incidents, and client satisfaction and turns them into fixes. Build these systems small and real from the start; it is far easier to grow a disciplined two-caregiver agency into a large one than to impose discipline on a chaotic one later.

Part 7 — Finding Clients & Referrals

Two engines fill your schedule: families who pay privately, and the aging-services network that steers people to agencies. Build both from the start.

- **Hospital discharge planners and case managers** send people home who need help — be the licensed agency they trust to prevent a readmission.
- **Skilled nursing and rehab facilities** discharge clients who still need in-home support after therapy ends.
- **Hospices** refer both ways: they need aides for personal care, and you refer declining clients to them.
- **Assisted living and senior communities** have residents who need one-on-one help beyond what the community provides.
- **The Aging and Disability Resource Connection (ADRC) and Area Agencies on Aging** route Oregon families and Medicaid clients to services.
- **Elder-law attorneys and geriatric care managers** guide families to trustworthy licensed agencies.

Your PolicyReady Referral System for Oregon lists named hospitals, skilled nursing facilities, hospices, senior communities, elder-law attorneys, and care managers across Portland, Salem, Eugene, Bend, and Medford, with the playbook for turning them into a steady stream of clients.

Referral relationships are built the same way everywhere: show up, be reliable, and make the referrer look good. Introduce your licensed agency to discharge planners, case managers, and community liaisons; give them a simple one-page overview and an easy way to reach you; and then deliver flawlessly on the first few referrals, because your handling of them is your real marketing. Follow up, close the loop on how a shared client is doing, and never make a referrer regret sending someone to you. In a licensed market, referral sources are choosing agencies they can trust with their own reputations — be the one that never gives them a reason to worry, and the referrals compound.

Part 8 — Getting Paid

In-home care in Oregon is paid three main ways: privately by the client or family; by the Oregon Health Plan (Oregon's Medicaid program) through Aging and People with Disabilities; and by other sources such as long-term-care insurance and Veterans benefits. Most new agencies start with private pay — Oregon is a premium market — and add Medicaid once established.

Private pay

Set a clear hourly rate, put it in a written service agreement, and bill only for services actually delivered. Private pay needs no enrollment and is the fastest way to open once you are licensed. Oregon's lack of a sales tax and its premium market can support healthy private-pay rates — build in your real labor costs, including overtime and Oregon's regional minimum wage, from the start.

The Oregon Health Plan (Medicaid) and APD

Oregon's Medicaid program is the Oregon Health Plan (OHP). In-home care for eligible older adults and people with disabilities is funded through Aging and People with Disabilities (APD) and the Home and Community-Based Services program, and an in-home care agency is one of the service options a Medicaid client can use. To serve Medicaid clients, you enroll as an OHP / APD provider — a process separate from your IHCA license — complete the required client screening and disclosure, provide only authorized services, and accept the Medicaid payment as full payment for covered services. Medicaid-funded in-home care is not available to a client living in a licensed community-based care setting or while the client is in a hospital or nursing facility.

Think about payer mix deliberately. Private pay is faster to start, pays more per hour, and gives you scheduling flexibility, but it depends on families who can afford it. Medicaid (OHP/APD) opens a much larger client base and steadier referral flow through case managers, but pays set rates and adds authorization and documentation requirements. Many successful Oregon agencies start private-pay to get licensed and stable, then layer on OHP/APD to grow volume, and later add long-term-care insurance and Veterans' benefits. Whatever the mix, price from your real costs — caregiver wages at your region's minimum or above, overtime, sick time, workers' compensation, insurance, supervision, and office overhead — and revisit your rates every year as Oregon's regional minimum wage rises.

A word on wages and overtime

Oregon's minimum wage varies by region and rises over time, with different rates for the Portland metro, standard counties, and non-urban counties — so confirm the rate for your area and update it each year. There is no state overtime law for this work, so the federal FLSA governs: pay overtime after 40 hours in a week, and remember that as a third-party employer you cannot claim the "companionship" exemption. Build these costs, plus Oregon sick time and workers' compensation, into your rates from day one.

Part 9 — Your First 90 Days

1. **Before you apply:** Choose your class, form the LLC, get your EIN and bank account, name your administrator, and bind insurance and workers' comp.
2. **Before you apply:** Adopt your policy manual, client forms, and caregiver handbook to Division 536, and build your screening and training systems.
3. **Apply (≥ 60 days out):** Submit your application and class fee through the OHA portal; begin caregiver background checks and registry enrollment.
4. **During review:** Complete caregiver orientation, initial and medication training, and competency evaluations; prepare for the survey using OHA's checklist.
5. **After licensure:** Take your first private-pay clients, run real supervisory oversight, begin OHP/APD enrollment, and start referral outreach to two or three hospitals or hospices in your region.

Part 10 — Common Mistakes to Avoid

- **Licensing at the wrong class.** If you plan to help with medications, license at Basic or above — providing a service above your class is operating outside your license.
- **Serving clients before the license issues.** You cannot provide in-home care before OHA issues your license; plan your timeline around it.
- **Letting a caregiver start before screening is complete.** No approved background check and registry status means no client contact.
- **Thin training or competency records.** Undocumented orientation, training, or competency is a survey finding waiting to happen.
- **Treating Medicaid enrollment as automatic.** OHP/APD enrollment is a separate process from your license — start it separately.
- **Underpricing.** Build wage, overtime, sick time, workers' comp, insurance, and supervision into your rate from the start.
- **Providing skilled care beyond your class.** Nursing services require the Comprehensive class and an Oregon-licensed RN — otherwise refer it out.

Part 11 — Staying Compliant & Growing

Once you are open, compliance is ongoing: renew your license on time; keep every caregiver's background check, registry status, training, and competency current; complete supervisory oversight and keep service plans updated; maintain insurance and business filings; and run a quality program that tracks complaints, incidents, and satisfaction and fixes what they show. Keep a compliance calendar so nothing lapses, and stay survey-ready at all times.

Growth comes from three moves: deepen referral relationships until you are the agency a hospital or hospice calls first; add payers (layer OHP/APD onto private pay, then long-term-care insurance and VA benefits); and, when you are ready, apply to move up a license class — for example from Basic to Comprehensive — so you can serve higher-need clients and widen your market. In a licensed state, your clean survey record and your reputation are the whole game.

As you grow, protect what got you there. Do not let recruiting pressure push you into hiring caregivers you have not fully screened and trained, do not take clients whose needs exceed your class or your capacity, and do not let documentation slip as volume rises — the failures that end agencies almost always trace back to compromising on screening, scope, or records to chase growth. Add caregivers and clients at a pace your administrator and supervision systems can actually support, keep your compliance calendar current, and reinvest in training and retention. Sustainable growth in Oregon looks like a widening base of loyal referral sources and a stable, well-supported caregiver team — not a spike in clients your systems cannot safely carry.

Part 12 — What It Costs to Start

Your startup budget has a few big categories. The largest fixed, known cost is your OHA application fee, which is set by your license class — \$2,000 (Limited), \$2,250 (Basic), \$2,500 (Intermediate), or \$3,000 (Comprehensive); verify the current amount with OHA. Beyond the fee, plan for the following, and gather real quotes rather than guessing, because they vary widely by region and by how you operate:

- **Insurance and bonding:** professional and general liability, workers' compensation, and optionally a bond or crime coverage protecting clients against theft — get quotes for your size and class.
- **Business formation:** LLC or corporation registration with the Oregon Secretary of State, plus any local business registration and your accounting setup.

- **Policies and documents:** your policy and procedure manual, client forms, and caregiver handbook built to Division 536 — the foundation your license and survey rest on.
- **Caregiver screening and training:** background checks, registry, orientation, initial and (for Basic and above) medication training, and competency evaluation for every caregiver.
- **Office and systems:** a small office or home office, scheduling and documentation software, phones, and basic supplies — in-home care is not equipment-heavy, which keeps startup lighter than facility-based care.
- **Working capital:** enough cash to cover several months of wages, insurance, and overhead before revenue is steady — this is the cost new owners most often underestimate, especially given the licensing timeline.

Oregon's lack of a statewide sales tax and the relatively low equipment needs of in-home care make it a comparatively accessible licensed business to start — but the working-capital runway to your first license and first clients is real, so fund it deliberately rather than assuming revenue will arrive quickly.

Two Worked Examples

Example 1 — A Basic-class private-pay start in Portland

You license at the Basic class so you can help clients with medication reminding and assistance as well as personal care and household support. You form an LLC, name a qualified administrator, bind insurance and workers' comp, and adopt your policy manual. You hire two caregivers, clear their Oregon background checks, enroll them on the registry, put them through four-hour orientation, initial training, and four-hour medication training, and evaluate their competency. You apply to OHA at least 60 days out, pass the survey, and receive your license. Your first client comes from a hospital discharge planner who needs safe help for a patient going home after a fall — you assess the home, write a service plan, and start daily personal care plus medication reminders, private pay, billed weekly. Referral targets you would build in Portland include hospitals, skilled nursing and rehab facilities, hospices, and senior communities across the metro.

Example 2 — Adding the Oregon Health Plan in Eugene or Salem

Your agency has a steady private-pay base and you are ready for Medicaid. You enroll as an Oregon Health Plan / APD provider — separate from your IHCA license — and complete the required screening and disclosure. A local APD or Area Agency on Aging case manager authorizes in-home care for an eligible client; you assign a screened, trained, competency-evaluated caregiver, deliver only authorized services per the service plan, supervise the case, and accept the Medicaid payment as full payment. As you take Medicaid clients, you keep the private-pay side growing through the same referral network — hospitals, skilled nursing, hospices, assisted living, and the ADRC — in Eugene, Salem, and the surrounding communities.

Who to Call & What to Ask

For...	Who to contact
Forming your business	Oregon Secretary of State, Corporation Division (sos.oregon.gov). Ask: what do I file to register my LLC, and what are the annual requirements?
Licensing your agency	Oregon Health Authority, Health Care Regulation and Quality Improvement — the licensing portal at hfclicensing.oregon.gov . Ask: what does a complete In-Home Care Agency application require for my class, and what are the current fees and timeframes?
Background checks	The Oregon Background Check Unit. Ask: how do my owners, administrator, and caregivers complete criminal records checks and get an approved determination?
Caregiver registry & training	OHA in-home care resources. Ask: how do caregivers enroll on the registry, and what orientation, training, and competency does Division 536 require for my class?
Becoming a Medicaid provider	Oregon Health Plan / Aging and People with Disabilities (oregon.gov/odhs). Ask: how do I enroll as an in-home care provider, and how does authorization work?
The aging network	The Aging and Disability Resource Connection (ADRC) of Oregon — 1-855-673-2372 — and your local Area Agency on Aging. Ask: how do families and Medicaid clients find and choose in-home care?
Wage & hour rules	Oregon Bureau of Labor and Industries (oregon.gov/boli) and the U.S. Department of Labor. Ask: what are the current regional minimum wage, sick time, and FLSA home care overtime rules?
Reporting abuse	Oregon Adult Protective Services through the ADRC / local APD office (report suspected abuse of a vulnerable adult under ORS 124); 911 for emergencies.

Key Terms

- **In-home care agency (IHCA):** a licensed Oregon agency providing personal care, household, medication, and (by class) nursing services in the home.
- **OAR Chapter 333, Division 536:** the Oregon Health Authority rules that govern in-home care agencies.
- **License class:** Limited, Basic, Intermediate, or Comprehensive — the class that sets your scope of services and fee.
- **Background Check Unit (BCU):** the Oregon unit that conducts caregiver criminal records checks and fitness determinations.
- **Caregiver registry:** the Oregon registry on which caregivers are enrolled and verified.
- **Competency evaluation:** the observation-plus-testing check a caregiver passes before serving independently.
- **Service plan:** the written plan describing a client's services, reviewed with each caregiver before care.
- **Oregon Health Plan (OHP):** Oregon's Medicaid program, which funds in-home care through Aging and People with Disabilities.
- **APD / ADRC:** Aging and People with Disabilities, and the Aging and Disability Resource Connection that routes Oregonians to services.

Core Rules & References

- OAR Chapter 333, Division 536 — Oregon Health Authority In-Home Care Agency licensing rules (OAR 333-536-0000 et seq.)
- ORS 443.305 to 443.355 — the in-home care agency statutes
- OAR 333-536-0007 — license classification (Limited, Basic, Intermediate, Comprehensive)
- OAR 333-536-0070 — caregiver qualifications, orientation, training, and competency
- OAR 333-536-0075 & 0080 — medication services and nursing services by class
- OAR 333-536-0093 — criminal records checks
- OAR 333-027 — Home Health Agency licensing (the separate, skilled category)
- ORS 124 — reporting abuse of a vulnerable adult; the FLSA home care rule and Oregon wage, sick-time, and workers'-compensation law

This guide is a general educational resource for prospective Oregon in-home care agency owners. It is not legal, tax, or licensing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with the agency named — the Oregon Secretary of State, the Oregon Health Authority, the Background Check Unit, and the relevant payer — before you rely on it. Provisions cited reflect Oregon law and OHA rules at the time of writing.